

Algorithme de Naranjo

Naranjo Algorithm

<u>Questions</u>	<u>Yes</u>	<u>No</u>	<u>Don't know</u>	<u>Score</u>
1. Are there previous conclusive reports on this reaction?	+1	0	0	1
2. Did the adverse event appear after the suspected drug was administered?	+2	-1	0	2
3. Did the adverse reaction improve when the drug was discontinued or a specific antagonist was administered?	+1	0	0	1
4. Did the adverse reaction reappear when the drug was re-administered?	+2	-1	0	0
5. Are there alternative causes (other than the drug) that could on their own have caused the reaction?	-1	+2	0	2
6. Did the reaction reappear when a placebo was given?	-1	+1	0	0
7. Was the drug detected in the blood (or other fluids) in concentrations known to be toxic?	+1	0	0	0
8. Was the reaction more severe when the dose was increased or less severe when the dose was decreased?	+1	0	0	1
9. Did the patient have a similar reaction to the same or similar drugs in any previous exposure?	+1	0	0	0
10. Was the adverse event confirmed by any objective evidence?	+1	0	0	1
			<u>TOTAL</u>	8

(Naranjo CA et al. "A method for estimating the probability of adverse drug reactions". Clin. Pharmacol. Ther. August 1981)

The Adverse Drug Reaction is assigned to a probability category from the total score as follows:

Definite	> 8
Probable	5 to 8
Possible	1 to 4
Doubtful	< 1

For inter-rater agreement, Kappa ranges from 0.69 to 0.86, which is equivalent of 83% to 92% agreement. Intra-rater reliability ranges from 80% to 97%, $k = 0.64$ to 0.95 . The inter-rater agreement was maintained on retesting, and results of the same order were obtained when physicians rated a different set of prospectively collected cases of Adverse Drug Reactions, demonstrating consensual, content and concurrent validity.